

Standard Operating Procedure

Medication Administration — Patient, Order and Timing Verification

«Hospital Name» — Supports MOM.7.a (Stop-Work), MOM.7.c (Stop-Work), MOM.7.d (CORE, Stop-Work), MOM.7.e, MOM.7.f, MOM.7.g

SOP No.	[Hospital to define]	Version	1.0
Effective Date		Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» verifies the patient, the order, the strength, the route, and the timing before every medication administration.

2. Scope

Applies to every medication administration, in every clinical area.

3. Responsibility

Registered Nurse / Administering Doctor: carries out every verification step before administration, with no exception.

4. Procedure

1. Confirm you are a registered nurse or MBBS-minimum doctor, or hold a documented statutory basis, before administering any medication.
2. Verify the patient using two identifiers — full name and the unique identification number — before every administration. Bed number alone is never acceptable, even when the ward is quiet and the bed is familiar.
3. Verify the order is complete against all five parameters — name, strength, route, frequency, time — before administering. If any parameter is missing, defer administration and clarify with the prescriber; never adjust or guess the missing parameter yourself.
4. For high-risk medications, complete an independent two-staff verification, with both staff documenting the check separately.
5. Verify strength against the order immediately before administering; if a discrepancy is found, defer and clarify — never adjust at the bedside.
6. Verify route (and site, where applicable) against the order before administering.
7. Verify timing against the order; where the order uses a frequency code only, confirm and document the correct clock time from the hospital's standard schedule.
8. Document every administration immediately after giving it, in the uniform location, as a separate entry for each dose.

5. Stop-Work Authority

Do not administer medication if you are not a registered nurse, MBBS-minimum doctor, or otherwise authorised by statute.

Do not administer without completing two-identifier patient verification — bed number alone is never sufficient.

Do not administer against an incomplete order — defer and clarify instead of filling in the gap yourself.

Raising a verification gap before administering is never a disciplinary matter, for any staff member regardless of seniority.

6. Records

Supported by Records 1, 3, 4, 5, 6, 7, and 9 of the MOM.7 Required Records pack.

7. Related Documents

- Policy on Safe Administration of Medications (HCO/MOM/POL/07) — MOM.7 policy
- SOP — IV Line and Tubing Safety (MOM.7.h)

8. Review

Reviewed annually, and immediately after any administration-related adverse event.